



A SOOTHEMADE BOOK

The Long Caregiving Stretch

For the months and years after the first 90 days.

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the caregiver in month seven, who has stopped counting the days because the count became overwhelming.

You are still doing this. That is the work.

A note from Maya

The first 90 days were the shock. This is the long stretch.

You have settled into a rhythm. The appointments are routine. The medications are mostly stable. You and the medical team know each other. The family has its pattern.

What you may not have is sustainability. The person you are caring for has needs that do not go away. The caregiver work is now your second job, on top of your first. Your marriage may be strained. Your own health may be slipping. Your siblings may be doing too little or too much, in ways you have not had the energy to fight about.

This book is for the version of you doing this for the long haul. The structure to make it sustainable. The honest pages for the anger and the exhaustion. The reckoning page for the day you realize you cannot do this forever and the question of what comes next.

With care, Maya

This is not medical or legal advice. The clinical decisions in your loved one's care belong to them, the medical team, and you and your family acting on their behalf. Financial and legal decisions belong to you and the professionals working with you. This book does not name medications. The crisis-line page is at the back. Caregivers are at meaningfully elevated risk for depression, anxiety, substance use, and their own health collapse. Knowing this in advance is one of the only ways to prevent it.

Chapter 1: You Are Doing Real Work

This is a job.

It is unpaid. It is largely unseen. It does not have a title that adequately describes it. It does not have a contract or sick days or a clear end date. It is not what you signed up for. It is what you showed up for.

The work includes:

- **Medical advocacy.** Knowing the team, the medications, the conditions, the warning signs.
- **Logistics.** Appointments, transportation, paperwork, insurance.
- **Direct care.** The physical tasks, whatever they are.
- **Emotional labor.** Being present, holding their fear, holding yours.
- **Social work.** Managing visitors, family, friends.
- **Financial management.** Their finances and yours.
- **Project management.** The household systems your caregiving requires.

These are skills. They count, even when no one outside this house sees them. They are also tiring, even when you would not trade the role.

Three things you have learned how to do that you did not know how to do a year ago. Write them down somewhere. They are real changes in you. They will outlast this chapter.

Chapter 2: The Medical Team Evolves

By month four, your medical team is established. By month six, it has shifted at least once. By month twelve, you have probably switched a specialist, brought in home health, or considered hospice.

The team is not static. The needs change. The team should change with them.

A few principles for the ongoing relationship:

- Re-fill the contact list every six months. The names change, the numbers change.
- Re-confirm the medications every time they change. Track by purpose in your notes, never by name. The pharmacy keeps the names.
- Bring written questions to every appointment. The appointment is shorter than your questions.
- Ask for plain language. "Can you explain that in words I will remember in six hours?"
- If a member of the team is not working for your loved one, ask for a different one. You can switch.

A note on patient advocates. They exist in most larger hospitals and health systems, and they can be shockingly effective at resolving billing errors, insurance denials, and care coordination issues. Ask for one by name.

Chapter 3: The Monthly Coordination

Who is doing what, this month.

In the long stretch, the workload split shifts. Someone takes on more. Someone takes on less. Paid help comes and goes. Siblings travel. The loved one's condition changes. The coordination has to be reviewed regularly or it will fall to the default (which is usually: more to you).

A monthly coordination check, in your own notes:

- Primary caregiver this month.
- Backup caregiver this month.
- Paid help (if any): hours per week, what they do.
- Each sibling or family member: what they do, when.
- Volunteer or friend helpers.
- Gaps in coverage (what is not getting covered).

The gaps are the most important line. Most caregivers, when they write this down honestly, find that they are personally filling 70 to 90 percent of the gaps. That ratio is unsustainable.

The fix is rarely "do more myself." The fix is asking for specific help (see *The Field Guide to Asking for Help* from this catalog), or hiring more paid help, or having a real conversation with the sibling or family member who is not doing their share.

Chapter 4: Respite Is the Most Important Chapter

Sustainability requires actual time off.

The kinds of respite available to caregivers:

- A family member taking over for a stretch (a sibling for a weekend, an adult child for a vacation).
- Paid in-home help for a day or weekend.
- Adult day care (the loved one goes to a facility for the day, comes home in the evening).
- A short-stay at a facility (some assisted living and skilled nursing facilities offer "respite stays" of a few days to weeks).
- Hospice respite (a Medicare benefit in the US for those in hospice care, allows the patient to stay at a facility for up to 5 days).
- Specialized respite programs (some non-profits and disease-specific organizations offer respite for caregivers of people with certain conditions, e.g. Alzheimer's).

What to reserve, right now:

- The next time you will get a real day off.
- The next time you will get a real weekend off.
- The next time you will be gone overnight.
- The next vacation (real, more than three days).

If any of those are blank, the next 90 days will be harder than the previous 90 days.

What blocks most caregivers from using respite:

- Cost.
- Guilt.
- The loved one resists.
- They cannot find the right option.
- They cannot find someone to cover.
- They have not asked, because they are scared the answer is no.

A note: caregivers who use regular respite stay in the caregiving role longer. Caregivers who do not use respite often collapse and the loved one ends up in a higher level of care than was otherwise necessary. Respite is not selfish. It is the tool that keeps this sustainable.

Chapter 5: I Am Angry

This chapter is for you alone. Burn it or shred the page if you printed it.

You are angry. The anger may be at the loved one, the situation, the siblings, yourself, the medical system, the universe.

The anger is information.

The anger is not character. It is data about what is too much.

The things most adult-child caregivers are secretly angry at their loved one about (even though their loved one did not choose this either):

- The dependence.
- The way the loved one is no longer the parent they were.
- The way the loved one sometimes resents the help.
- The way the loved one's choices, made years ago (or never made), are now your problem.
- The way the loved one is still here, in a body that is failing, and you are watching it.

The anger does not make you a bad caregiver. It makes you a person doing a hard job. The anger needs somewhere to go besides your loved one. A therapist. A caregiver support group. A trusted friend. A walk. A pillow. Cold water on your face.

What you cannot do is bottle it forever. Anger that is bottled comes out in other places (the partner, the kids, the dog, the road rage, the wine bottle).

A note about caregiver rage: it is real, common, and underdiscussed. If you are seeing it in yourself, you are not alone, and there is help.

Chapter 6: I Am Exhausted

What exhaustion feels like, right now. The last time you felt rested, really.

For most long-haul caregivers, the answer to "when did you last feel rested" is "I cannot remember." Often: "before this started."

What you are running on:

- Coffee.
- Adrenaline.
- Sheer stubbornness.
- The fear of what happens if you stop.
- Real rest. (You probably cannot check this one.)

What you have given up that used to restore you. Which one could you get back, first, if you made it a priority. What it would take.

A reminder: exhaustion is not a personality trait. It is a sign that the work is bigger than the rest. The fix is rest, not trying harder.



Chapter 7: I Cannot Do This Forever

The hardest chapter in the book.

The thought you are not telling anyone. Most long-haul caregivers have this thought. It is not character. It is signal.

What the thought is actually about:

- "I cannot keep up this level of care."
- "I want my life back."
- "I am scared they will outlive my own health."
- "I am scared my marriage will not survive this."
- "I am scared I will not survive this."
- "I love them and I am also done."
- Something else specific to your situation.

What would have to change for you to be okay:

- More respite.
- More paid help in the home.
- A move to a different level of care for the loved one.
- A different sibling arrangement.
- Your own therapist or counselor.
- A real time-off plan.
- Acceptance of help you have been refusing.
- A different kind of conversation with the team about prognosis.

- All of the above.

The conversation you need to have:

- With your partner.
- With your sibling(s).
- With the medical team.
- With a therapist.
- With the loved one (if able).
- With yourself (the honest version).

A note: adult children who move a parent to a higher level of care are not failing. They are often making the decision the parent would have made for themselves, if the parent could still decide.

Partners who recruit more paid help are not giving up. They are recognizing that one person is not enough for this workload.

You are allowed to be honest about the limit, even while still loving them.

Chapter 8: The Sibling Conversations

For families with more than one adult child involved. Often the source of more exhaustion than the caregiving itself.

The workload division, in plain words. The honest accounting. Hours per week, by person. The imbalance, if there is one. The reason for the imbalance.

The old story your family tells about each of you. The one about you. The one about each sibling. Which old story is shaping the current workload.

What each person brings that the others do not. The acknowledgment that fairness in hours is not the only metric. A sibling 1,000 miles away cannot do the same hours as the local sibling. They can do other things.

The conversations you need to have, with each sibling. The thing you want them to know. The thing you want them to do differently. The thing you are willing to do differently.

The family meeting. When. Where. Who. The agenda you are bringing. Often: update on the loved one, current workload, the needs not being met, your proposal for redistribution, the questions for each sibling, the next 60-day commitments.

The backup plan if the meeting does not work. A family therapist or elder-care mediator. A written agreement, even if informal. Hiring more paid help. Stepping back yourself in a meaningful way.

A note about old wounds. Family dynamics from when you were younger often come back during eldercare. The favored child. The black sheep. The one who left. The one who stayed. None of those identities get to drive the current decisions, but they all show up in the room.

Some relationships do not survive a family caregiving episode. Some get stronger. Which it will be is not entirely up to you. What is up to you: how you show up for the conversations, how honest you are, how much you let the old family story decide who you are now.

Chapter 9: What They Would Want, If They Could Tell Me

For caregivers of loved ones with dementia or advanced illness. The decisions you make on their behalf are real decisions, weighted by what they would want.

What they used to say about getting old. What they used to say about medical intervention. What they used to say about nursing homes or facilities. What they used to say about funerals. What values they lived by.

The decision you have to make this month. What they would choose, in your best guess.

A note: you are not betraying them by making a hard choice. You are extending who they were into the decisions they can no longer make. This is the sacred part of caregiving.

Chapter 10: The Marriage / Partnership

Caregiving is a leading cause of marital strain and divorce. The work has to be done to keep the marriage standing.

An honest check on your marriage or partnership:

- How often do we have a real conversation per week.
- How often do we have one that is NOT about caregiving.
- How often are we physically affectionate.
- How often do we go out together (alone, no kids, no caregiving).

The thing you miss most about you-before-this. The thing your partner has been carrying that you have not acknowledged. The thing you have been carrying that your partner has not acknowledged.

What you could commit to, this month:

- One date night, real, away from the house and the loved one.
- One conversation per week that is just about us.
- Couples counseling (worth investing in, often).
- A weekend away (real respite, planned).
- Reading the same book together.
- Going to bed at the same time.

A note: couples counseling is the single best investment many caregiving couples make. It is not a failure. It is a maintenance tool for the partnership most likely to absorb this load.

Chapter 11: Your Job and Your Career

For caregivers still in paid work.

The current job situation. Hours per week, in paid work.

Flexibility you have. Manager who knows the caregiving situation. Family Medical Leave (US) used so far.

What you have given up at work. What you are not giving up, no matter what.

Options to consider:

- Family Medical Leave (intermittent or block, US).
- Reduced schedule.
- Switching to a more flexible role.
- A different employer with better caregiving support.
- A career pause (with eyes open to financial and re-entry implications).
- Leveraging employer's Employee Assistance Program (EAP).

A reminder: HR professionals who handle FMLA requests have seen every caregiving situation. They are often helpful allies, not adversaries. Ask specifically: "What options do I have for caregiving leave?"

Chapter 12: Your Own Health

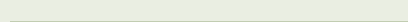
Caregivers neglect their own medical care. This is well-documented and a leading cause of caregiver health collapse.

When was your last primary care visit. Dental cleaning. Eye exam. Mammogram or prostate exam (if relevant age). Colonoscopy (if relevant age). Therapy session. Real exercise. Real meal sitting down. Real sleep night.

Any appointments you have been delaying. Any symptoms you have been ignoring.

The next three appointments you are scheduling. Write them down. Then schedule them.

A reminder: the loved one needs you to be well. You need you to be well. The appointments you are delaying are not optional. They are part of keeping this sustainable.



Chapter 13: Financial Caregiving and Hospice Considerations

The ongoing financial picture.

Monthly caregiving costs: paid help, facility, medications, medical equipment, transportation, specialized food, insurance copays, out-of-network expenses. Add them up. The annual total is often startling.

Long-term-care insurance status. Medicaid / Medicare status.

Hospice considerations. Many people qualify for hospice earlier than they realize. Hospice in the US is largely covered by Medicare. It is NOT just "the last week of life." It can be months of comfort-focused care, often at home, often dramatically improving the loved one's quality of life and the caregiver's burden.

The stigma around hospice ("giving up") is largely unfounded. Hospice is a comfort-focused care team. People sometimes stabilize or even improve once the focus shifts from curative to comfort. The caregiver burden often decreases meaningfully under hospice. Worth having the conversation early.

The question to bring to the medical team: is the loved one eligible for hospice? What would change if they were?

The answer may surprise you, in either direction.

Chapter 14: A Final Letter

You are doing one of the hardest things a human being does.

You are sustaining the life and dignity of someone who can no longer fully sustain themselves. You are doing it in a culture that does not always acknowledge the work. You are doing it on top of your own life, your own marriage if you have one, your own career if you have one, your own children if you have them. You are doing it for months, sometimes years, often without anyone saying "I see what you are doing and I am grateful."

I see what you are doing. I am grateful.

This book does not make this easier. It is a tool, not a cure. But it is also a record. A record of the time you took to think about this honestly. A record of the conversations you had the courage to have, with siblings, with partners, with yourself. A record of the moments you knew you were exhausted, knew you were angry, knew you could not do this forever, and still came back the next day.

When this chapter closes, whichever way it closes, you will look back on what you did. The book will still be here. You will read what you wrote in your hardest moments and you will be amazed at the person who showed up.

You are still that person. You are still showing up.

The book closes here. The work continues. So do you.

With care, Maya

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents and the people taking care of them.

The printable companion to this book is the **Family Caregiver Workbook** at notes.soothemade.com, with monthly trackers and re-printable templates.

Also from Soothemade

- **Caregiving, the First 90 Days**, the prequel to this book.
 - **The Field Guide to Asking for Help**, a foundational tool for the long stretch.
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Caregiver crisis lines

United States

- 988, Suicide and Crisis Lifeline (call or text).
- 1-800-677-1116, Eldercare Locator (Area Agency on Aging, caregiver support + respite).
- 1-800-445-8106, Family Caregiver Alliance.
- 1-800-272-3900, Alzheimer's Association 24/7 Helpline (for dementia and other dementias).
- 1-800-662-4357, SAMHSA National Helpline (substance use, free, confidential, 24/7).

United Kingdom

- 0808 808 7777, Carers UK.
- 116 123, Samaritans.

Canada

- Carers Canada (find local programs).
- 9-8-8, Suicide Crisis Helpline.

Australia

- 1800 422 737, Carer Gateway.

A reminder: caregivers often delay seeking help until they are in crisis. The earlier you involve a therapist, a support group, or a crisis line, the better the outcomes. This is not weakness. This is professional athletes having trainers. The work is hard. The supports are the tools that make it possible.

This book is for planning and personal use only. It is not medical or legal advice.

A Soothemade book. Made slowly, in plain language.